Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

PERI-OPERATIVE AND POST-OPERATIVE MANAGEMENT

Anaesthetist Sedation Guideline

YG CG 406 Feb 2022

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FIRST APPROVED: APRIL 2019 REVIEW BY: FEB 2025

Key Points

- Sedation when performed correctly is a safe and useful technique.
- Deep sedation is considered to require the same level of care as for general anaesthesia.
- Anaesthetists' involvement in sedation requires the same level of care as for general anaesthesia.

	Minimal Sedation/ Anxiolysis	Moderate Sedation/ Analgesia ('Conscious Sedation')	Deep Sedation/ Analgesia
Responsiveness	Normal response to verbal stimulation	Purposeful* response to verbal or tactile stimulation	Purposeful* response following repeated or painful stimulation
Airway	Unaffected	No intervention required	Intervention may be required
Spontaneous Ventilation	Unaffected	Adequate	May be inadequate
Cardiovascular function	Unaffected	Usually maintained	Usually maintained
Escalation of required competencies			

* Reflex withdrawal from a painful stimulus is NOT considered a purposeful response. Excerpted from Continuum of Depth of Sedation. Definition of General Anesthesia and Levels of Sedation/ Analgesia of the American Society of Anesthesiology. From the ASA, 520N, Northwest Highway, Park Ridge, Illinois, 60068-2573, USA.

Conduct

- It is not anticipated that anaesthetists would generally be involved in minimal or conscious sedation.
- If anaesthetists are involved in sedation (because of our familiarity and ability to use the deeper end of the continuum) the same level of care as for a GA should be applied.
- Prepare the patient with appropriate fasting, preoperative assessment and consent including the requirement for a responsible adult that night if the patient is a day case.
- The anaesthetist is responsible for this sedation throughout the procedure.
- Preparation of equipment, monitoring, drugs and presence of appropriately trained anaesthetic assistant should all meet AAGBI and RCoA standards regardless of location.
- Full monitoring including NIBP, SPO_2, 3 lead ECG and capnograph is mandatory.
- Access to the patient should be optimised and advanced airway equipment and resuscitation equipment must be immediately available.
- There must be good communication between the anaesthetist and the procedure operator e.g. knowing when a dental surgeon is going to proceed to inject local anaesthesia.
- The anaesthetist is responsible until the patient is stable in the recovery area and the recovery staff have been given a full handover. The anaesthetist must ensure the recovery and postoperative management standards are the same as for GA.

Sedation Technique

- As with general anaesthesia sedation must be tailored to the individual patient and the procedure undertaken.
- If there is an element of pain then analgesia and/or local anaesthesia should be given.
- The effects of a combination of opioids and benzodiazepines are synergistic. Great care should be taken as individuals vary in their response especially with age. This risks inadvertently producing deep sedation or general anaesthesia in those patients who are most vulnerable.
- Flumazenil and Naloxone should be available, but their use should be a last resort, not routine and it should be noted that their effect wears off after a short period of about 20 minutes. Use of these reversal agents should be recorded as excessive use is considered an indicator of poor quality care.
- As per department guidelines no anaesthetist should commence sedation or anaesthesia if they do not have the appropriate staffing or equipment or if they are not able to provide this service for the individual patient and procedure safely. If necessary they should contact the Consultant Anaesthetist on call for help.

References

Chapter 19 Guidelines for the Provision of Anaesthesia Services (GPAS) Guidance on the Provision of Sedation Services 2016

Safe Sedation Practice for Health Care Procedures: Standards & Guidance, Academy of Medical Colleges 2013

National safe sedation course RCoA 2018